Slicepoint POC: Reconstruction Spec

Nov 9, 2026 · @John Kalafut

Purpose and provenance

Slicepoint answers one question for a hospital: is this AAA segmentation model a fit for our patients and scanners before we turn it on? It returns a GO / NO-GO verdict backed by five data-health checks scored 0 to 100, with drill-downs and a one-page PDF.

- **Origin:** student team POC built on Base44 at the Nucleate hackathon, co-presented problem with Aneurisk.
- **Rights:** team owns copyright and IP under Nucleate terms; agreed next steps are under an MIT license. Get that grant in writing before code or design lands near FORGE.
- **Source:** captured from the live app at unselfish-safe-scan-check.base44.app on 2026-11-09. Minified Vite/React bundle, no source maps, Base44 backend. No code export was available on the team's tier.
- **Stance:** this is a spec, not a codebase. Screens, checks, thresholds and copy carry over; the implementation does not.
- **Data:** the demo runs on made-up scans (100 scans, 100 people, "Hospital A" against itself). Nothing here is a real performance result.

Route map and screens

Nine routes in two audiences: plain-language screens for decision makers, and one technical tab for clinicians and analysts. Every chart card follows the same pattern: title, one-line definition, Ours vs What the AI learned from, a plain sentence with the headline number, a "What this means" line, and a "Show the numbers" disclosure.

Route	Audience	What it shows
`/` Overview	Exec / committee	GO or NO-GO verdict with a FIT badge and one-sentence rationale. Five health-check cards scored 0 to 100 against a target marker, each naming its weakest sub-factor. PDF builder: pick tabs (Patients, Scans and equipment, Labels and measurements, AI accuracy, For doctors, After go-live); verdict and health check always included. Demo banner.
`/details` Patients	Exec	Ours vs reference distribution bars for sex, age group (10-year bins), aneurysm size class (none, 30 to 54 mm, 55 mm or more). Each category flagged Covered or not.
`/details/scans` Scans and equipment	Exec	Scans vs distinct people (leakage check), scanner brand, contrast timing (venous, arterial, none), slice thickness (thin 1 mm or less, standard, thick).
`/details/labels` Labels and measurements	Exec	Aneurysm prevalence vs reference. Where this hospital draws the aneurysm line on a 20 to 40 mm scale against the 30 mm standard. Histogram of aorta diameters in 5 mm bins with a dashed 30 mm line.
`/details/accuracy` AI accuracy	Exec	Dice gauge with 0.81 basic and 0.90 clinical markers. Measured size: share of scans within an adjustable tolerance (default 3 mm) that recomputes the page. Aneurysms caught / missed / missed with no AI answer. Worked / didn't / can't tell bars per subgroup against a 90% line.

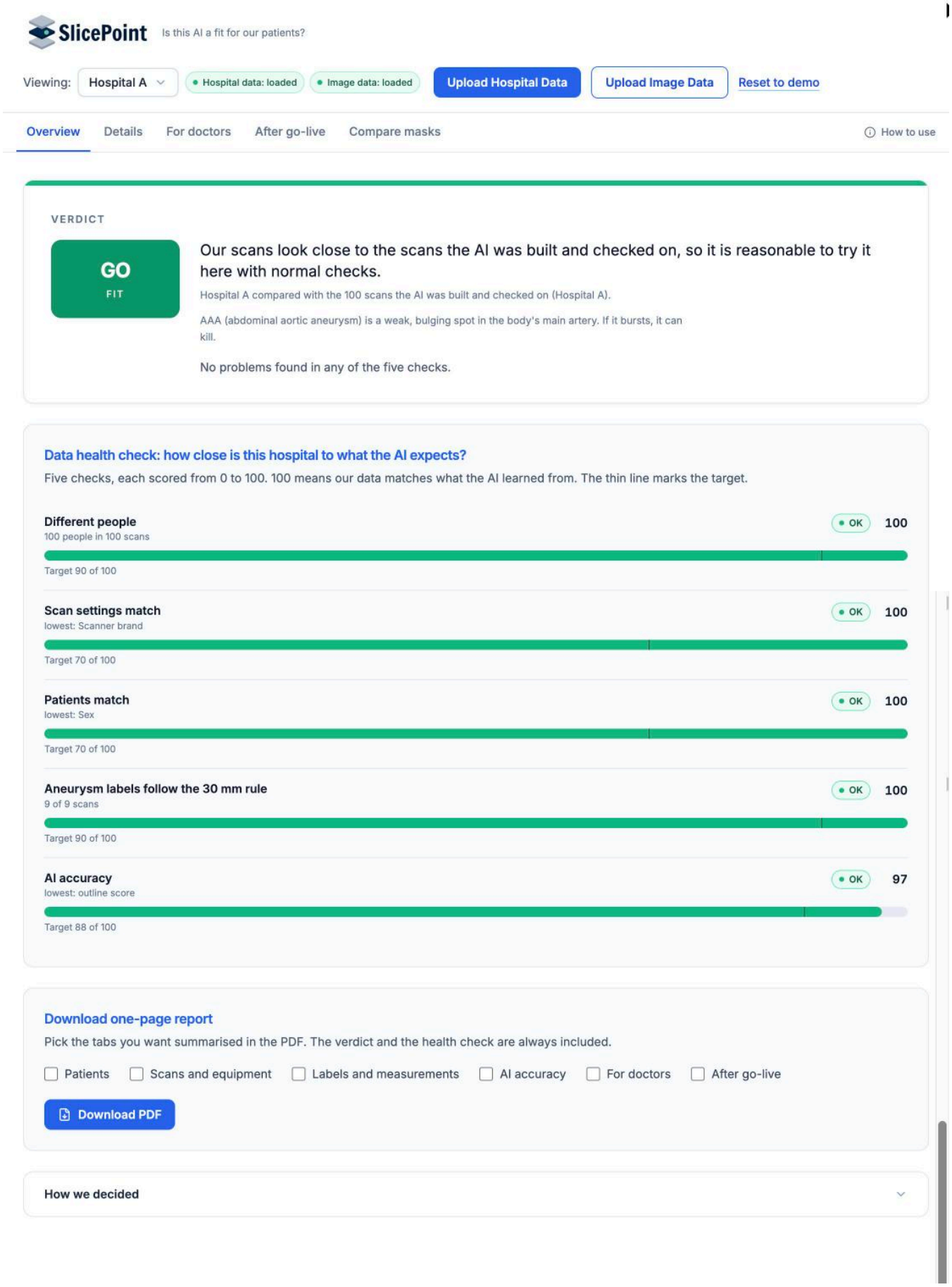
Route	Audience	What it shows
`/for-doctors` Technical detail	Clinician / analyst	Cohort completeness. Sensitivity and specificity two ways (missing counts as wrong; when answered) with 95% Wilson CIs, balanced accuracy, G-mean, size-class confusion matrix. Bland-Altman plot colored by slice thickness. Decision-line analysis at 30 and 55 mm. Stratified table (bias, MAE, within tolerance, sensitivity; "Too few" suppression). Segmentation summary. Cases to review list. Six methods-and-limits statements.
`/after-go-live`	Operations	Monthly check-ins after deployment. Empty until a monthly summary JSON is uploaded.
`/compare-masks`	Demo	Pick a built-in synthetic CT image and vendor mask, render overlay on canvas, show a preset Dice. No model runs.
`/how-to-use`	All	Notebook-to-upload workflow, file naming, privacy statement, troubleshooting table, glossary.

The bundle also ships Base44 auth routes (`/login` , `/register` , `/forgot-password` , `/reset-password` , `/mfa`) that the app does not use.

Baseline screenshots

The student team's POC as it ran on 2026-11-09, demo data loaded, one full-page capture per screen. This is the baseline the rebuild starts from and the reference for what the team designed. The Base44 edit badge and the mouse cursor were removed; nothing else was changed.

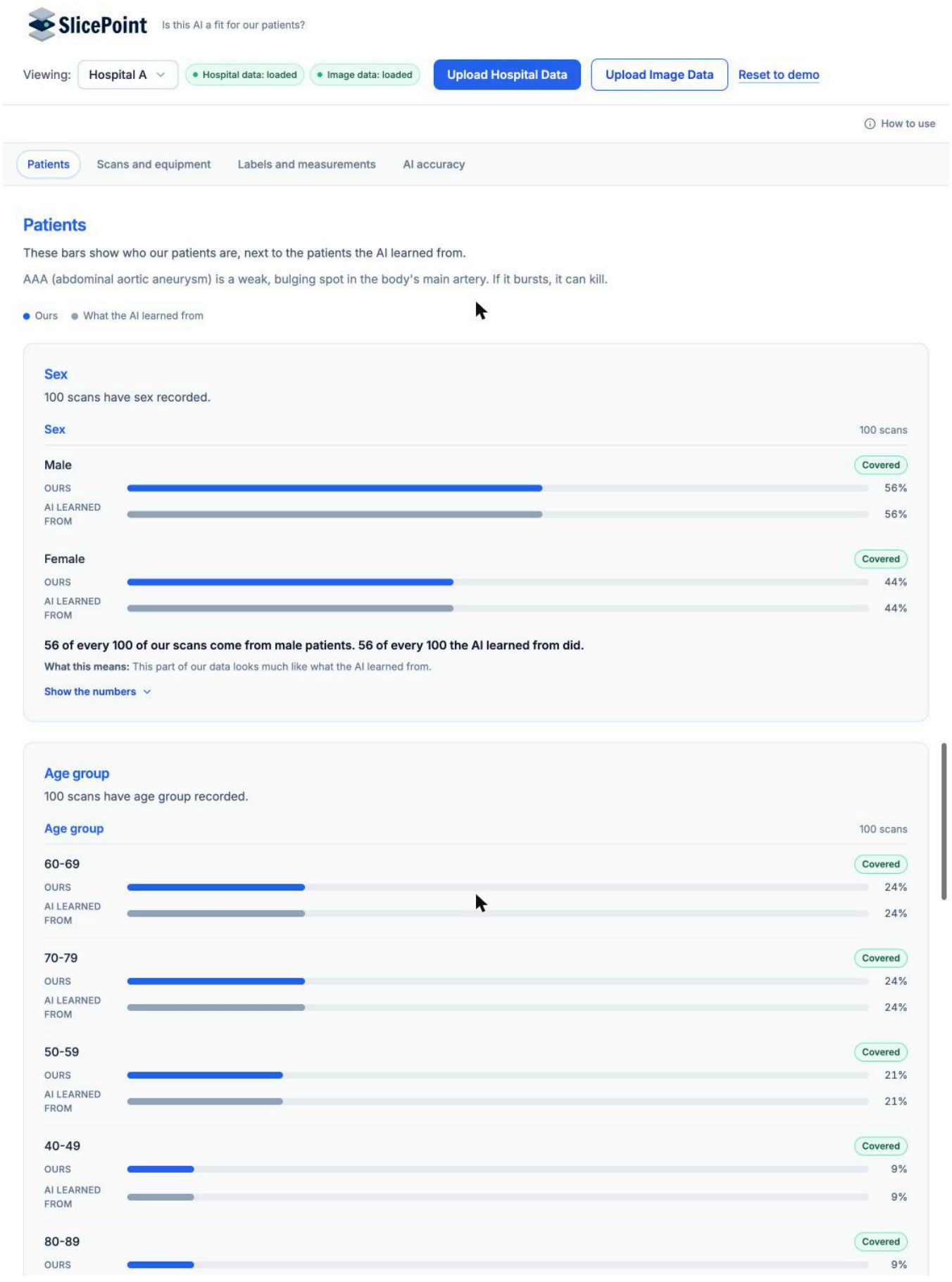
Overview

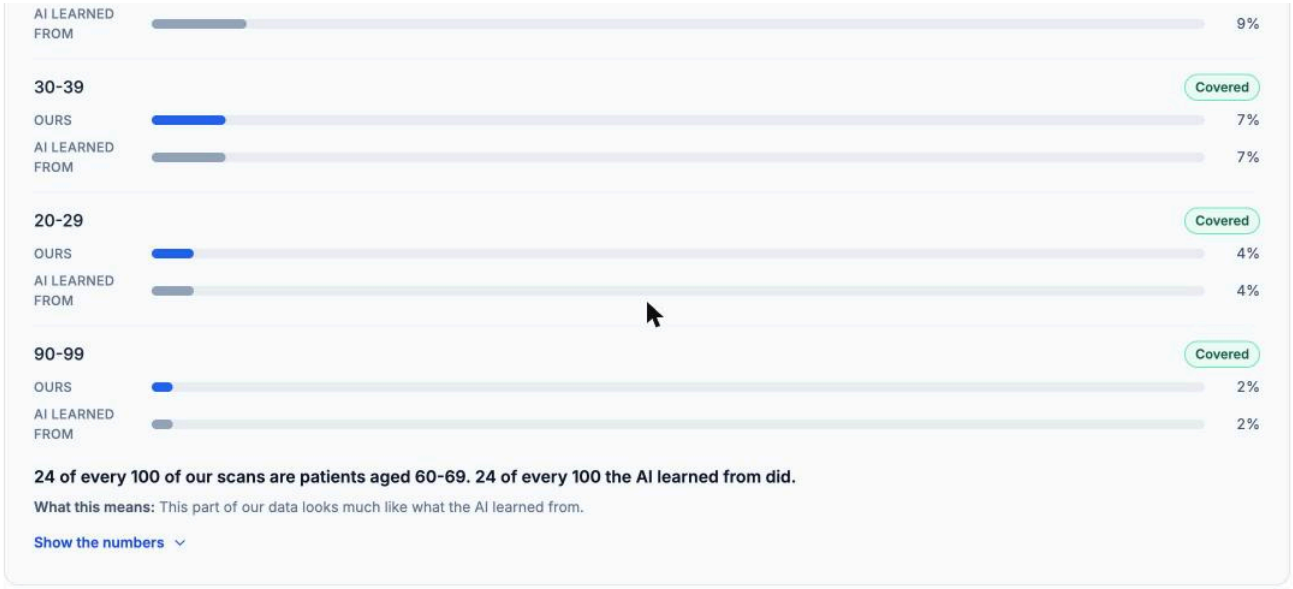

SlicePoint
Is this AI a fit for our patients?
Viewing:
Hospital A
Hospital data: loaded
Image data: loaded
Upload Hospital Data
Upload Image Data
Reset to demo
Overview
Details
For doctors
After go-live
Compare masks
How to use
VERDICT
GO
FIT
Our scans look close to the scans the AI was built and checked on, so it is reasonable to try it here with normal checks.
Hospital A compared with the 100 scans the AI was built and checked on (Hospital A).
AAA (abdominal aortic aneurysm) is a weak, bulging spot in the body's main artery. If it bursts, it can kill.
No problems found in any of the five checks.
Data health check: how close is this hospital to what the AI expects?
Five checks, each scored from 0 to 100. 100 means our data matches what the AI learned from. The thin line marks the target.
Different people
100 people in 100 scans
OK
100
Target 90 of 100
Scan settings match
lowest: Scanner brand
OK
100
Target 70 of 100
Patients match
lowest: Sex
OK
100
Target 70 of 100
Aneurysm labels follow the 30 mm rule
9 of 9 scans
OK
100
Target 90 of 100
AI accuracy
lowest: outline score
OK
97
Target 88 of 100
Download one-page report
Pick the tabs you want summarised in the PDF. The verdict and the health check are always included.
Patients
Scans and equipment
Labels and measurements
AI accuracy
For doctors
After go-live
Download PDF
How we decided

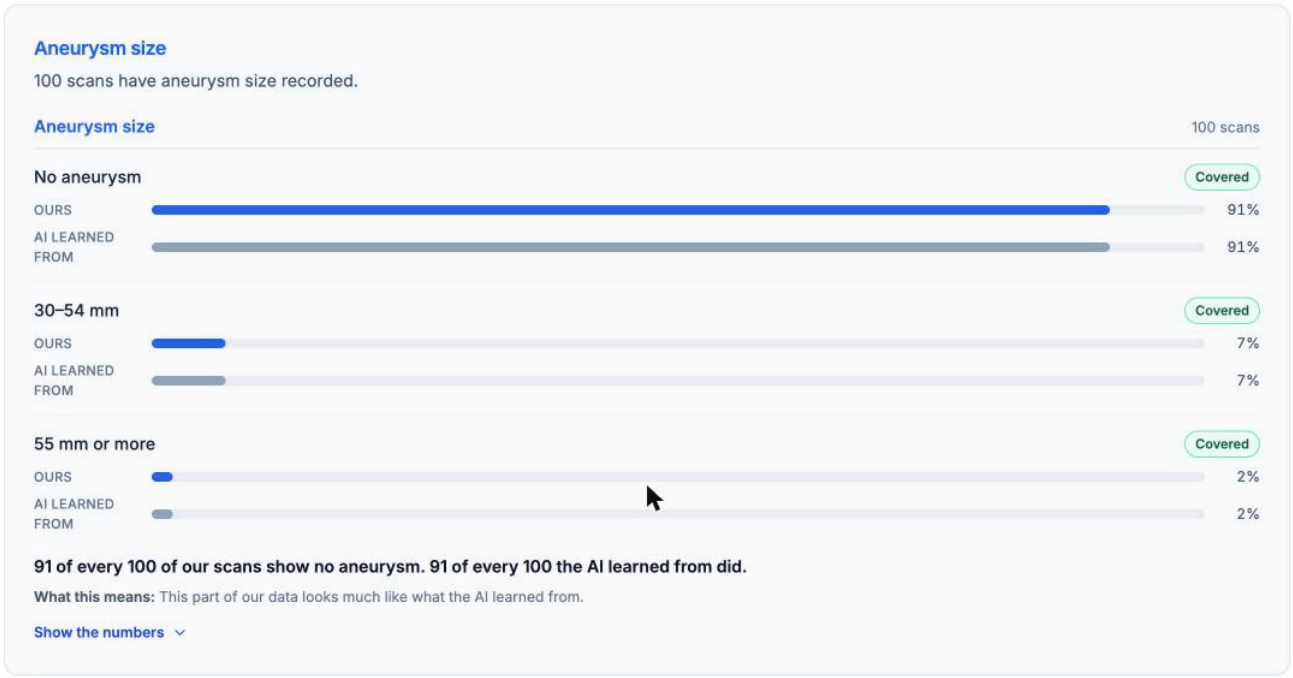
Results come from a test sample, not a full trial. This report supports a decision and is not medical advice.

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Details: Patients


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How to use
Patients
Scans and equipment
Labels and measurements
AI accuracy
Patients
These bars show who our patients are, next to the patients the AI learned from.
AAA (abdominal aortic aneurysm) is a weak, bulging spot in the body's main artery. If it bursts, it can kill.
Ours
What the AI learned from
Sex
100 scans have sex recorded.
Sex
100 scans
Male
Covered
OURS
56%
AI LEARNED FROM
56%
Female
Covered
OURS
44%
AI LEARNED FROM
44%
56 of every 100 of our scans come from male patients. 56 of every 100 the AI learned from did.
What this means: This part of our data looks much like what the AI learned from.
Show the numbers
Age group
100 scans have age group recorded.
Age group
100 scans
60-69
Covered
OURS
24%
AI LEARNED FROM
24%
70-79
Covered
OURS
24%
AI LEARNED FROM
24%
50-59
Covered
OURS
21%
AI LEARNED FROM
21%
40-49
Covered
OURS
9%
AI LEARNED FROM
9%
80-89
Covered
OURS
9%

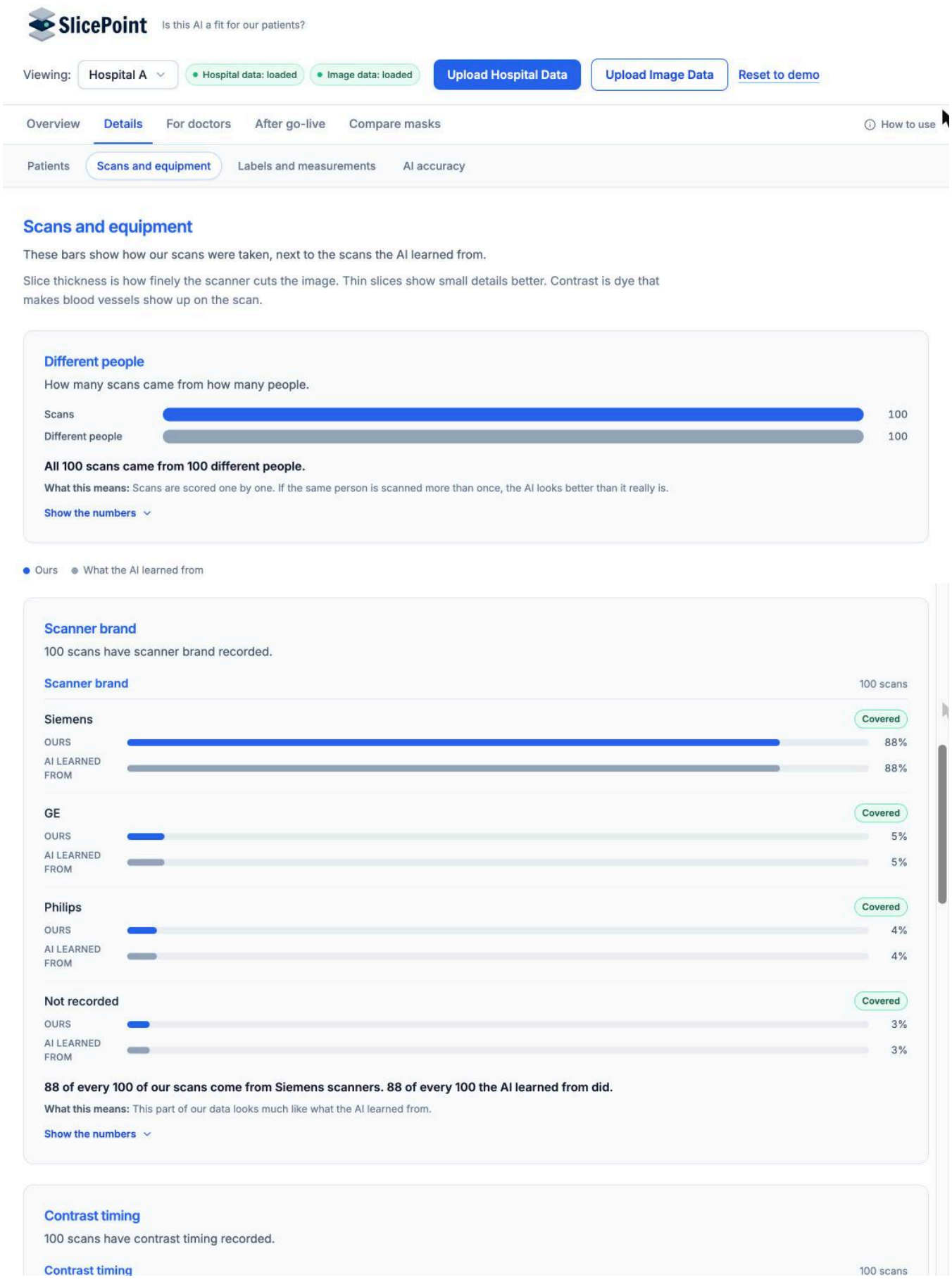


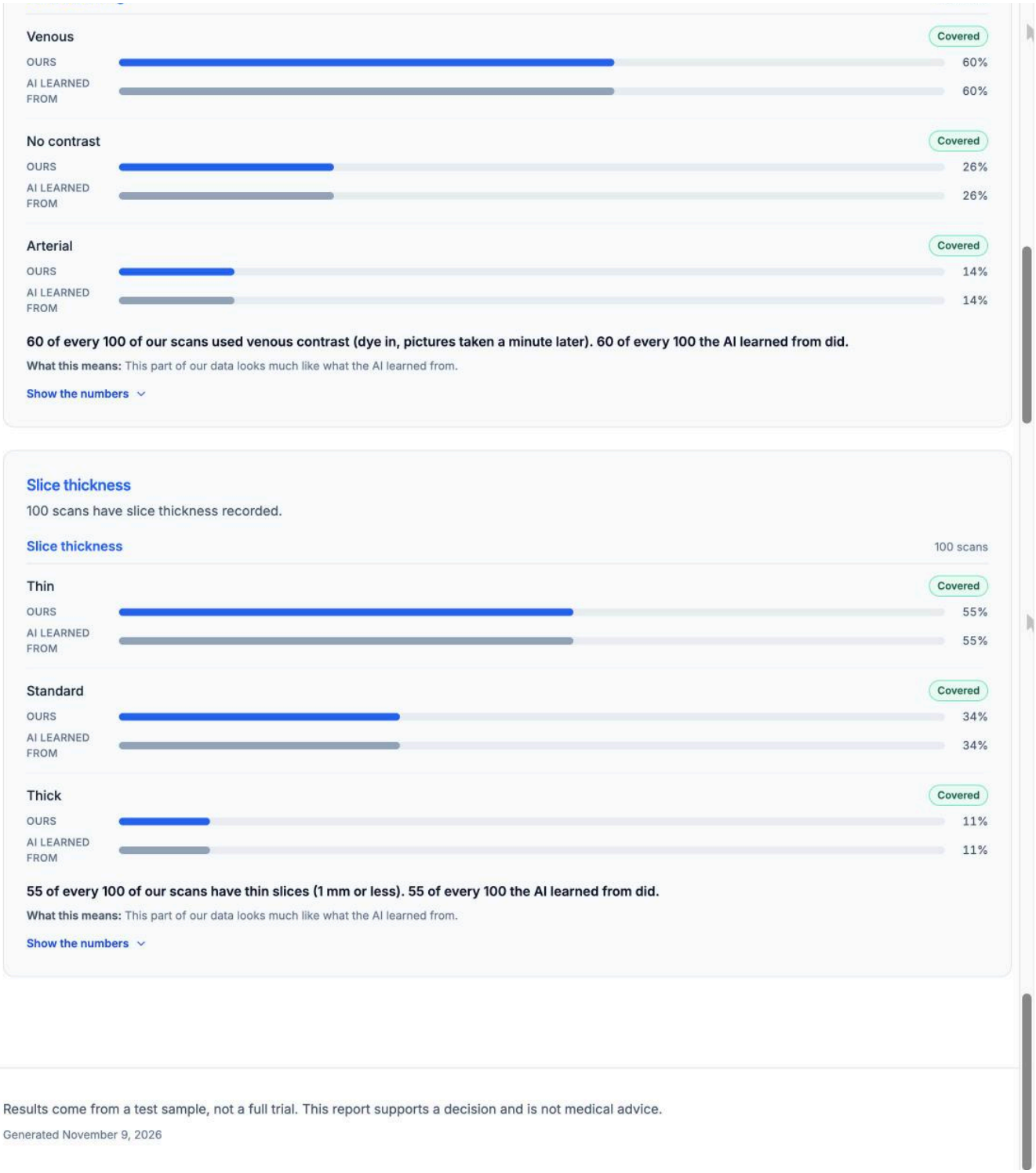


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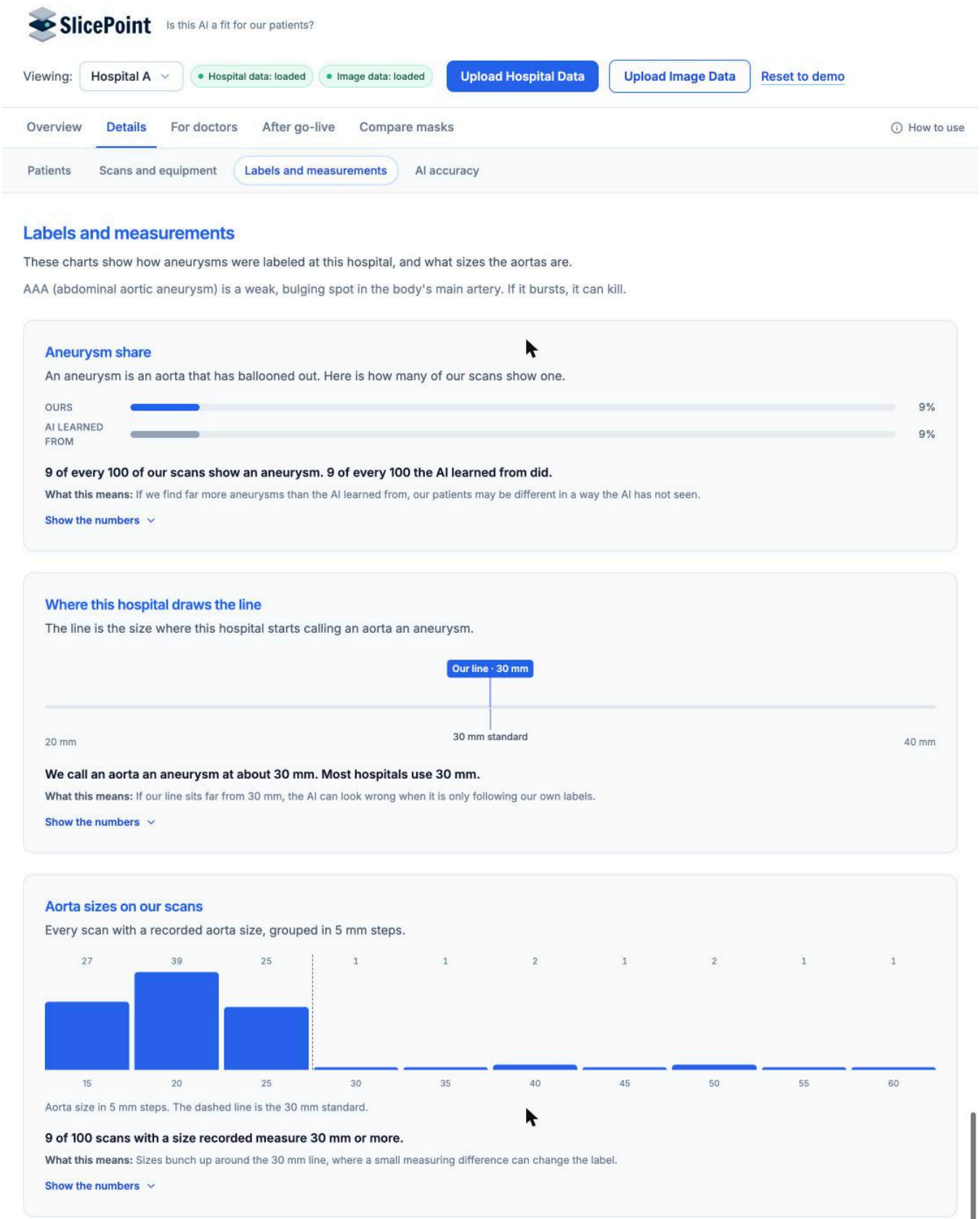
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Details: Scans and equipment


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How to use
Patients
Scans and equipment
Labels and measurements
AI accuracy
Scans and equipment
These bars show how our scans were taken, next to the scans the AI learned from.
Slice thickness is how finely the scanner cuts the image. Thin slices show small details better. Contrast is dye that makes blood vessels show up on the scan.
Different people
How many scans came from how many people.
Scans
100
Different people
100
All 100 scans came from 100 different people.
What this means: Scans are scored one by one. If the same person is scanned more than once, the AI looks better than it really is.
Show the numbers
Ours
What the AI learned from
Scanner brand
100 scans have scanner brand recorded.
Scanner brand
100 scans
Siemens
Covered
OURS
88%
AI LEARNED FROM
88%
GE
Covered
OURS
5%
AI LEARNED FROM
5%
Philips
Covered
OURS
4%
AI LEARNED FROM
4%
Not recorded
Covered
OURS
3%
AI LEARNED FROM
3%
88 of every 100 of our scans come from Siemens scanners. 88 of every 100 the AI learned from did.
What this means: This part of our data looks much like what the AI learned from.
Show the numbers
Contrast timing
100 scans have contrast timing recorded.
Contrast timing
100 scans



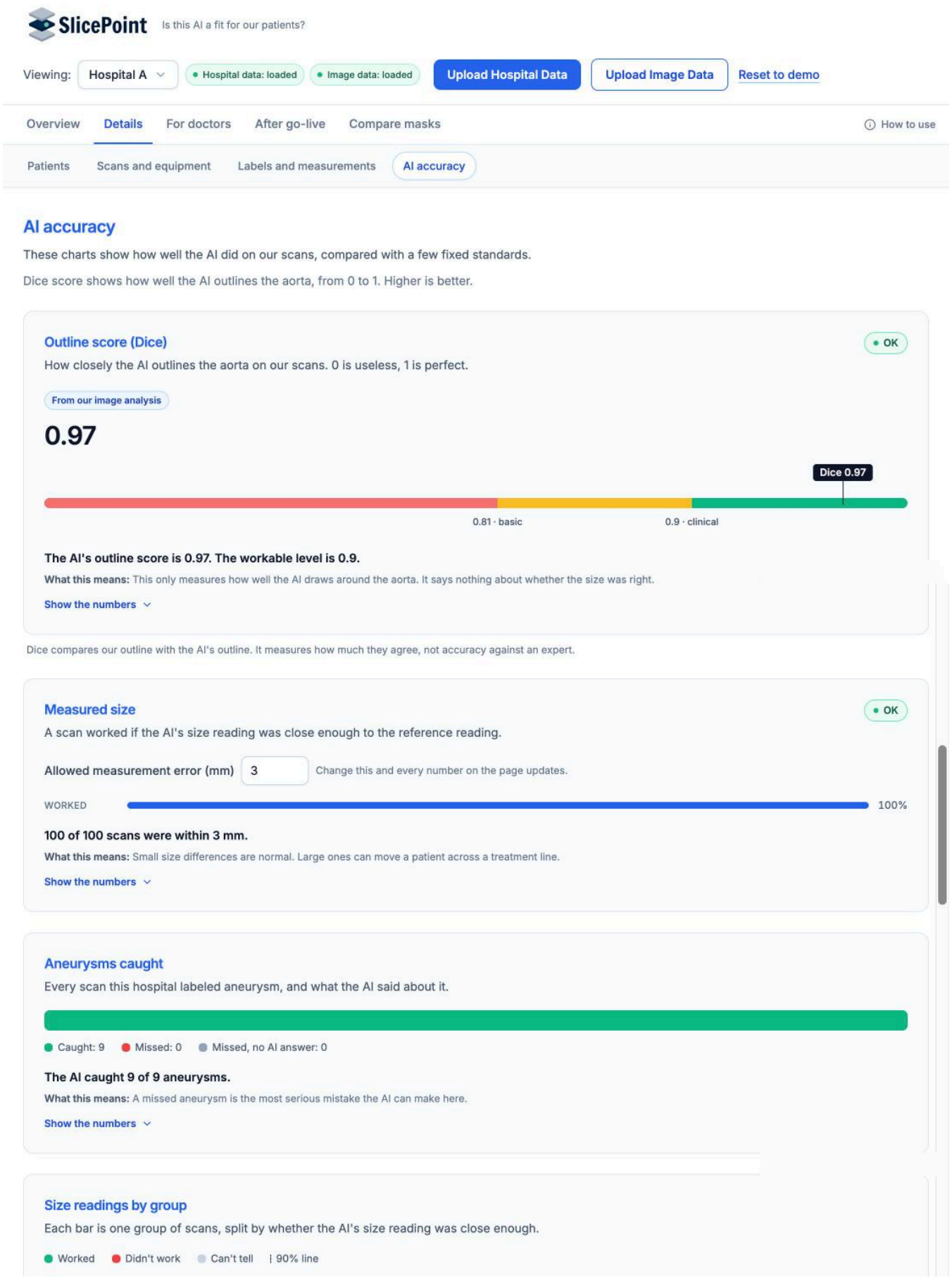
Details: Labels and measurements


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How to use
Patients
Scans and equipment
Labels and measurements
AI accuracy
Labels and measurements
These charts show how aneurysms were labeled at this hospital, and what sizes the aortas are.
AAA (abdominal aortic aneurysm) is a weak, bulging spot in the body's main artery. If it bursts, it can kill.
Aneurysm share
An aneurysm is an aorta that has ballooned out. Here is how many of our scans show one.
OURS
9%
AI LEARNED FROM
9%
9 of every 100 of our scans show an aneurysm. 9 of every 100 the AI learned from did.
What this means: If we find far more aneurysms than the AI learned from, our patients may be different in a way the AI has not seen.
Show the numbers
Where this hospital draws the line
The line is the size where this hospital starts calling an aorta an aneurysm.
Our line · 30 mm
20 mm
30 mm standard
40 mm
We call an aorta an aneurysm at about 30 mm. Most hospitals use 30 mm.
What this means: If our line sits far from 30 mm, the AI can look wrong when it is only following our own labels.
Show the numbers
Aorta sizes on our scans
Every scan with a recorded aorta size, grouped in 5 mm steps.
27
39
25
1
1
2
1
2
1
1
15
20
25
30
35
40
45
50
55
60
Aorta size in 5 mm steps. The dashed line is the 30 mm standard.
9 of 100 scans with a size recorded measure 30 mm or more.
What this means: Sizes bunch up around the 30 mm line, where a small measuring difference can change the label.
Show the numbers

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Details: AI accuracy


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AI accuracy
AI accuracy
These charts show how well the AI did on our scans, compared with a few fixed standards.
Dice score shows how well the AI outlines the aorta, from 0 to 1. Higher is better.
Outline score (Dice)
OK
How closely the AI outlines the aorta on our scans. 0 is useless, 1 is perfect.
From our image analysis
0.97
Dice 0.97
0.81 · basic
0.9 · clinical
The AI's outline score is 0.97. The workable level is 0.9.
What this means: This only measures how well the AI draws around the aorta. It says nothing about whether the size was right.
Show the numbers
Dice compares our outline with the AI's outline. It measures how much they agree, not accuracy against an expert.
Measured size
OK
A scan worked if the AI's size reading was close enough to the reference reading.
Allowed measurement error (mm)
3
Change this and every number on the page updates.
WORKED
100%
100 of 100 scans were within 3 mm.
What this means: Small size differences are normal. Large ones can move a patient across a treatment line.
Show the numbers
Aneurysms caught
Every scan this hospital labeled aneurysm, and what the AI said about it.
Caught: 9
Missed: 0
Missed, no AI answer: 0
The AI caught 9 of 9 aneurysms.
What this means: A missed aneurysm is the most serious mistake the AI can make here.
Show the numbers
Size readings by group
Each bar is one group of scans, split by whether the AI's size reading was close enough.
Worked
Didn't work
Can't tell
90% line


Sex
Male 56/56
Female 44/44
Age group
60-69 24/24
70-79 24/24
50-59 21/21
80-89 9/9
40-49 9/9
30-39 7/7
20-29 4/4
90-99 2/2
Aneurysm size
No aneurysm 91/91
30–54 mm 7/7
55 mm or more 2/2
Scanner brand
Siemens 88/88
GE 5/5
Philips 4/4
Not recorded 3/3
Contrast timing
Venous 60/60
No contrast 26/26
Arterial 14/14
Slice thickness
Thin 55/55
Standard 34/34
Thick 11/11
Male (Sex): 56 of 56 scans were within 3 mm.
What this means: A group that does worse than the rest tells us where the AI needs watching.
Show the numbers
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For doctors

SlicePoint Is this AI a fit for our patients?

Viewing: Hospital A · Hospital data: loaded · Image data: loaded · Upload Hospital Data · Upload Image Data · Reset to demo

Overview | Details | For doctors | After go-live | Compare masks | How to use

Technical detail

For doctors and analysts. This tab uses clinical and statistical terms. Every section says what its numbers mean and shows the values behind them.

Hospital A · 100 scans

Cohort structure

How much of the data is actually there before any scoring starts.

Aorta size recorded	100 of 100
AI gave an answer	100 of 100
Scanner brand recorded	100 of 100
Aneurysm label recorded	100 of 100

Scans per person: 1 to 1, median 1.

100 scans from 100 people. 0 scans have no AI answer and 0 have no reference measurement.

What this means: Missing values are handled differently by different measures, so it matters how many there are.

Show the numbers

Classification

Sensitivity is the share of labeled aneurysms the AI caught. Specificity is the share of normal aortas it called normal.

MISSING COUNTS AS WRONG		WHEN ANSWERED	
Sensitivity	1.00 (9/9), 95% CI 0.70 to 1.00	Sensitivity	1.00 (9/9), 95% CI 0.70 to 1.00
Specificity	1.00 (91/91), 95% CI 0.96 to 1.00	Specificity	1.00 (91/91), 95% CI 0.96 to 1.00

Balanced accuracy: **1.00** · G-mean: **1.00**

SIZE CLASS, REFERENCE VS AI

Reference	AI: no aneurysm	AI: 30 to 54	AI: 55 or more	Total
No aneurysm	91	0	0	91
30 to 54 mm	0	7	0	7
55 mm or more	0	0	2	2

100 of every 100 scans land in the same size class (100 of 100).

Few aneurysm cases. Intervals are wide.

Sensitivity 1.00 (9/9), specificity 1.00 (91/91).

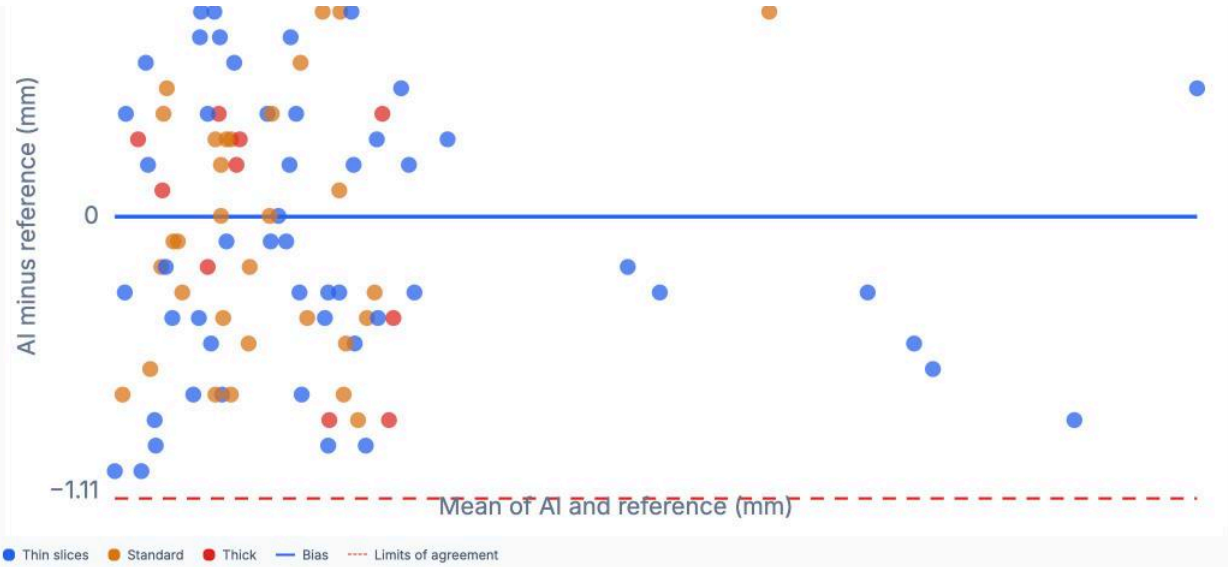
What this means: Missing AI answers count as wrong here, so both numbers describe what a patient would actually get.

Show the numbers

Measurement agreement

Each dot is one scan. The blue line is the average difference, the red dashed lines the limits of agreement.





The AI reads 0 mm smaller than the reference on average, and 100 of every 100 readings are within 2 mm.

What this means: A bias that is not zero means every reading is shifted the same way, which a hospital can correct for.

Show the numbers

Decision lines

Scans within 5 mm of each line, and how many the AI moves to the other side.

30 mm line — 26 scans nearby · 0 moved across

0 moved above the line, 0 moved below.

55 mm line — 3 scans nearby · 0 moved across

0 moved above the line, 0 moved below.

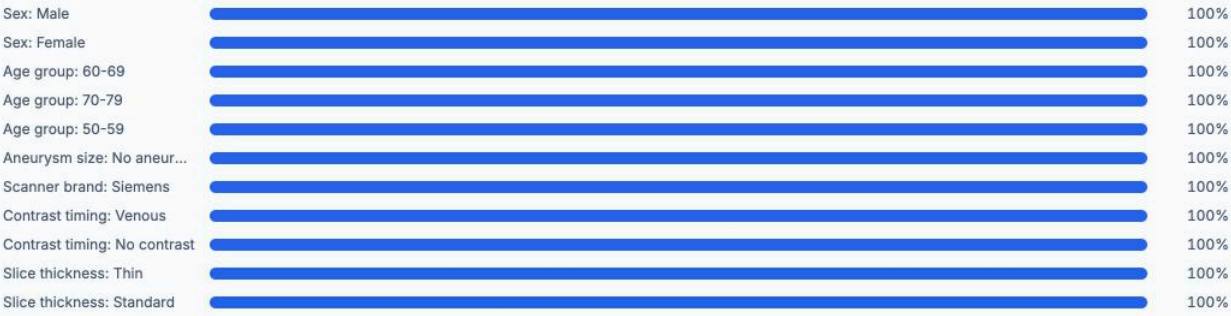
29 scans sit within 5 mm of a decision line. The AI moves 0 of them across.

What this means: A one-millimetre difference matters most for patients sitting on one of these lines.

Show the numbers

Stratified results

Every group is scored on its own. Small groups jump around by chance.



Group	Value	Scans	People	Bias	MAE	Within allowed	Sensitivity
Sex	Male	56	56	-0.03	0.49	100 of 100	1.00 (6/6)
Sex	Female	44	44	0.02	0.49	100 of 100	1.00 (3/3)
Age group	60-69	24	24	-0.1	0.55	100 of 100	1.00 (2/2)
Age group	70-79	24	24	-0.1	0.47	100 of 100	1.00 (1/1)

Age group	80-89	9	9	Too few	Too few	Too few	Too few
Age group	40-49	9	9	Too few	Too few	Too few	Too few
Age group	30-39	7	7	Too few	Too few	Too few	Too few
Age group	20-29	4	4	Too few	Too few	Too few	Too few
Age group	90-99	2	2	Too few	Too few	Too few	Too few
Aneurysm size	No aneurysm	91	91	0.01	0.49	100 of 100	—
Aneurysm size	30–54 mm	7	7	Too few	Too few	Too few	Too few
Aneurysm size	55 mm or more	2	2	Too few	Too few	Too few	Too few
Scanner brand	Siemens	88	88	0.02	0.5	100 of 100	1.00 (9/9)
Scanner brand	GE	5	5	Too few	Too few	Too few	Too few
Scanner brand	Philips	4	4	Too few	Too few	Too few	Too few
Scanner brand	Not recorded	3	3	Too few	Too few	Too few	Too few
Contrast timing	Venous	60	60	0.03	0.48	100 of 100	1.00 (6/6)
Contrast timing	No contrast	26	26	-0.07	0.54	100 of 100	1.00 (1/1)
Contrast timing	Arterial	14	14	Too few	Too few	Too few	Too few
Slice thickness	Thin	55	55	-0.03	0.51	100 of 100	1.00 (8/8)
Slice thickness	Standard	34	34	0.02	0.47	100 of 100	1.00 (1/1)
Slice thickness	Thick	11	11	Too few	Too few	Too few	Too few

Male (Sex): 100 of every 100 scans within the allowed error.

What this means: Each group is tested on its own. With many groups, some will look worse by chance.

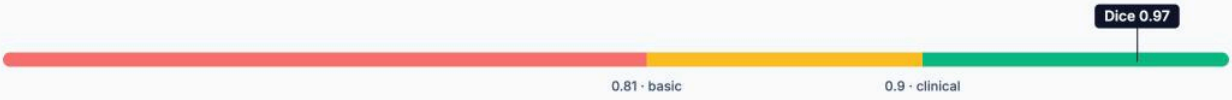
Show the numbers

Segmentation

Dice runs from 0 to 1. The lines mark 0.81 and 0.9.

From our image analysis

0.97



Below 0.90	—
Below 0.85	—

Dice compares our outline with the AI's outline. It measures how much they agree, not accuracy against an expert.

Dice mean 0.97, median —. The share below 0.90 is not in this file.

What this means: A low outline score means the shape is unreliable, so any size taken from it needs a second look.

Show the numbers

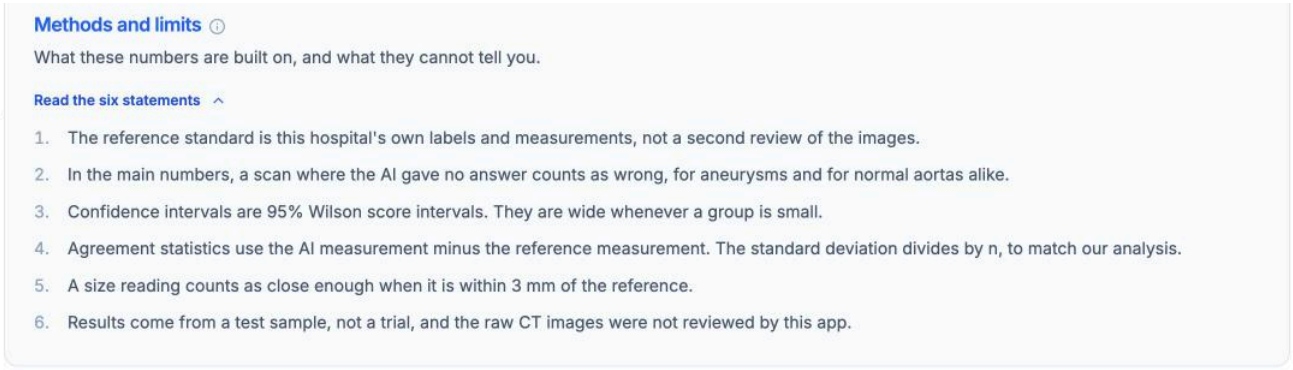
Cases to review

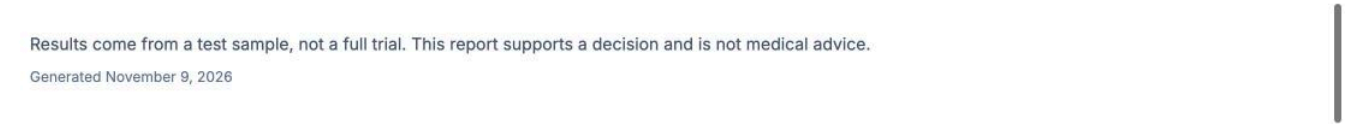
Highest difference first. Only the first 6 characters of each scan ID are shown.

Every scan agrees with its label and is inside the allowed error.

0 of 100 scans need a second look.

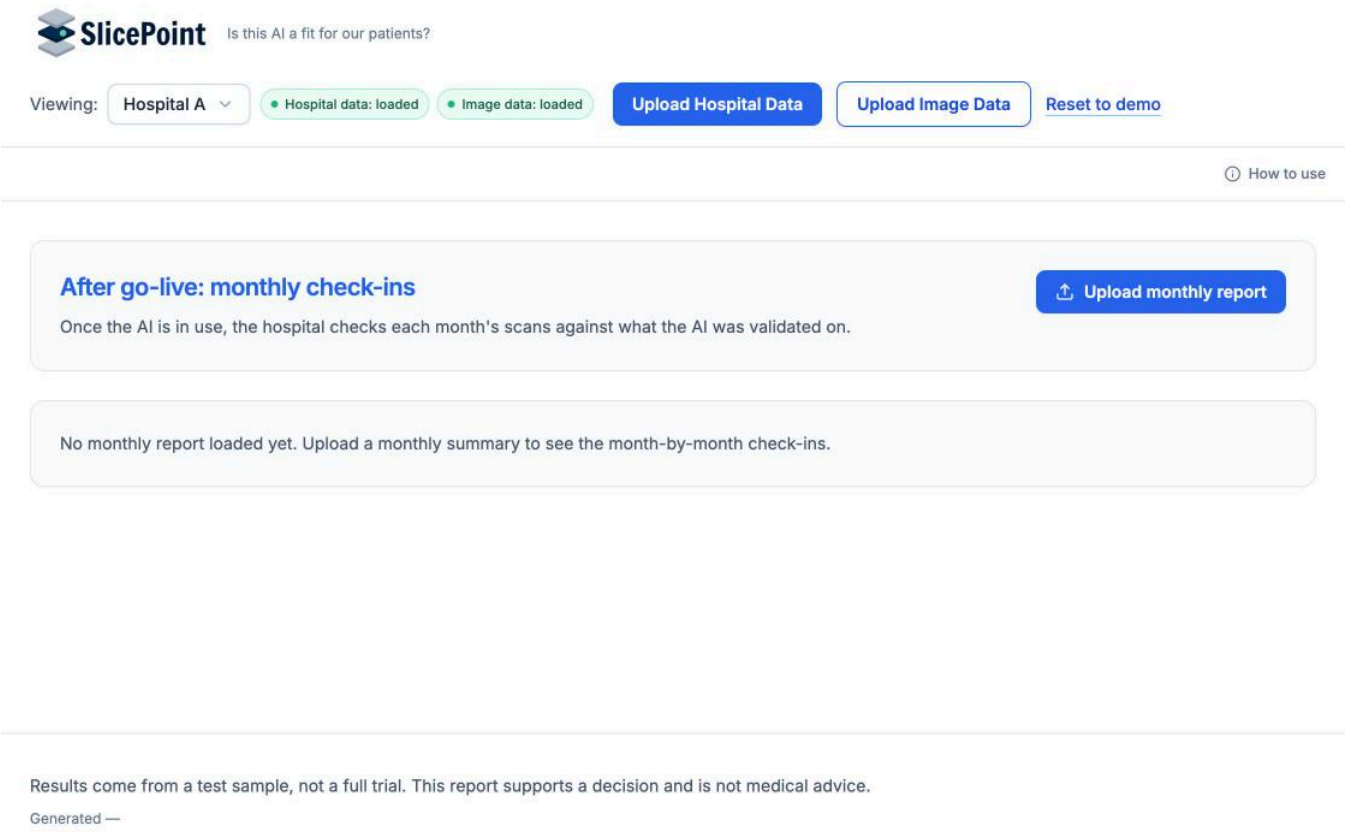
What this means: These are the scans where a person should check the images before anything is changed.



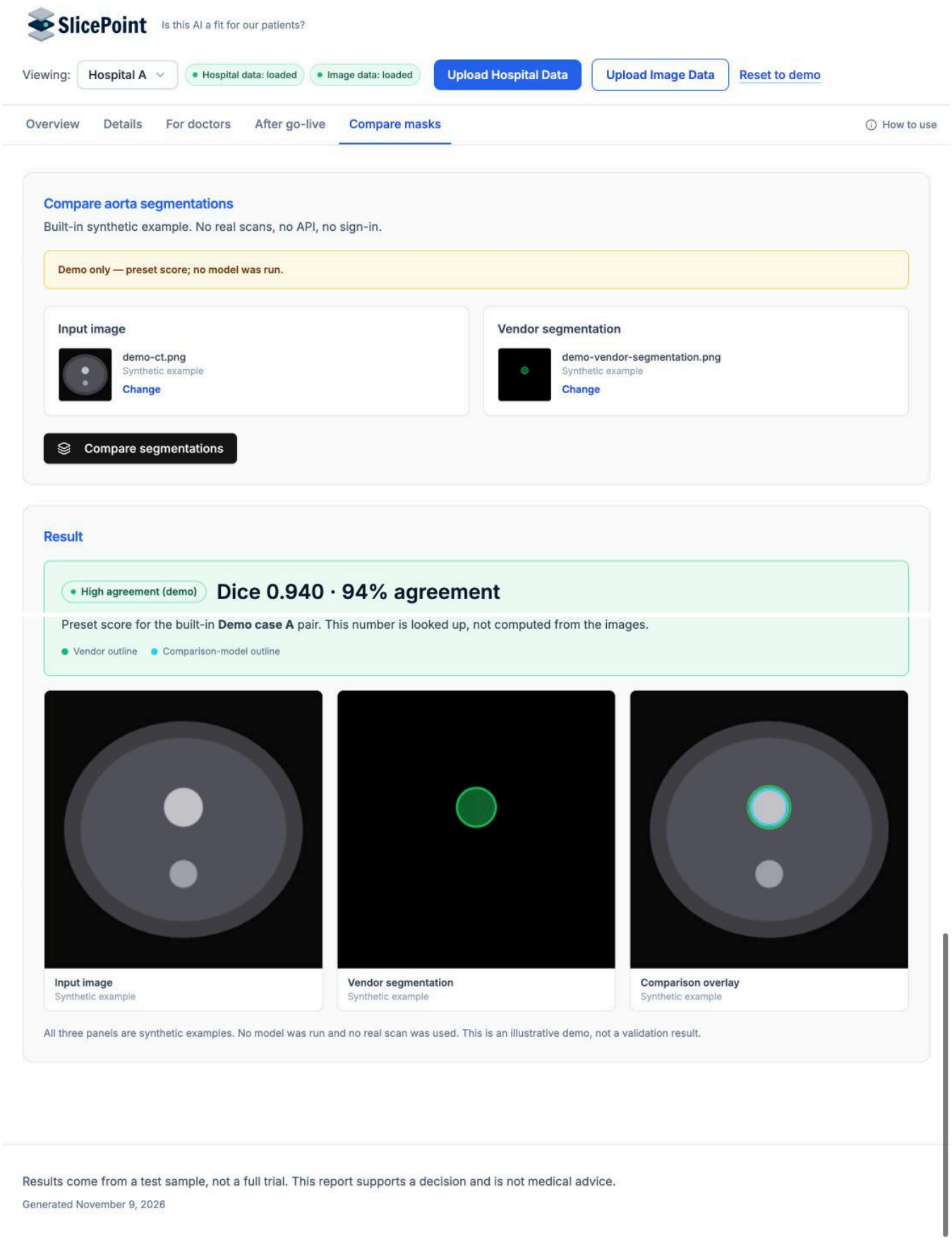


The Bland-Altman plot has no x-axis tick labels, and its axis title overlaps the lower limit line. Worth fixing in the rebuild.

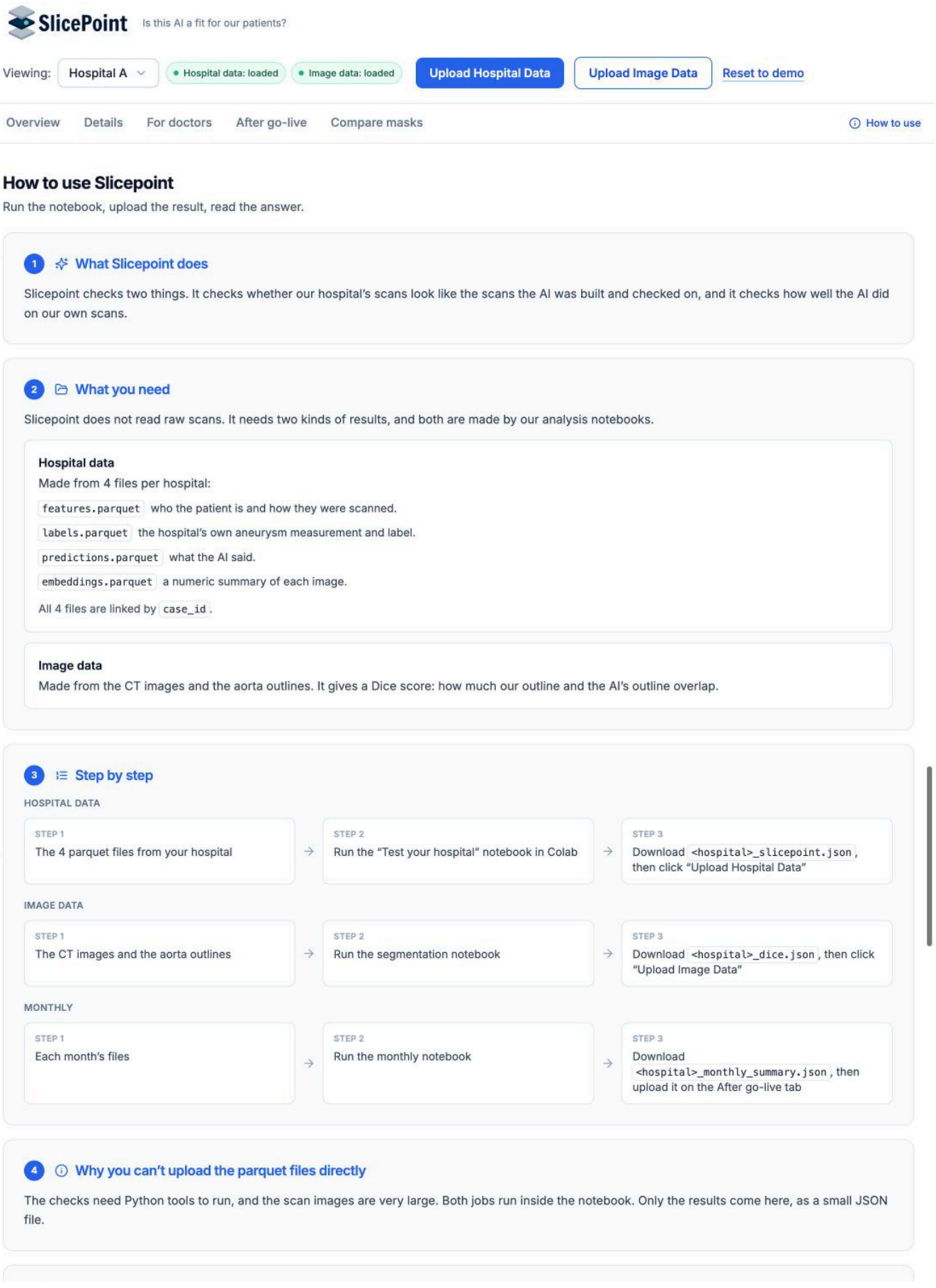
After go-live

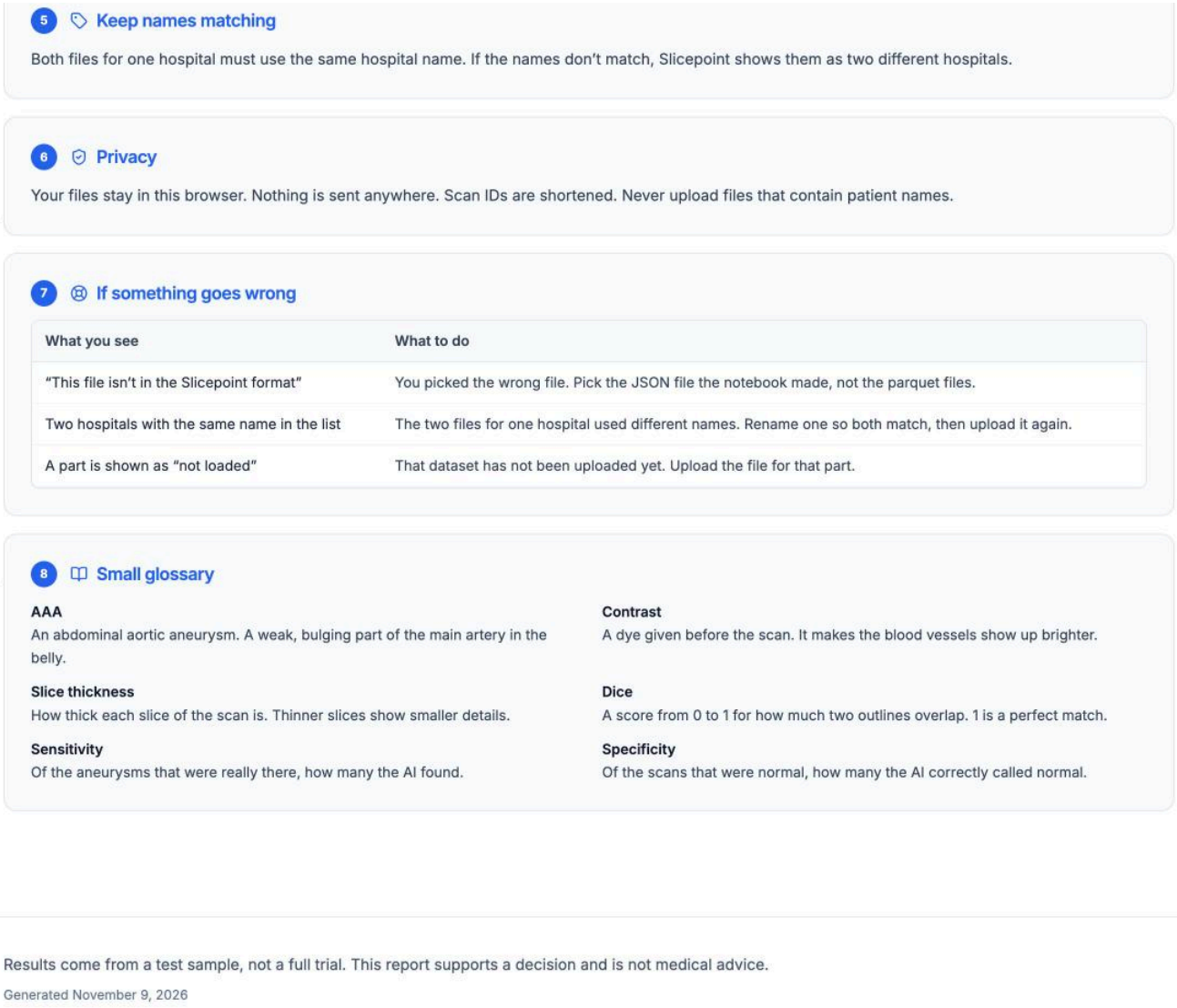


Compare masks (after running the demo pair)


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Image data: loaded
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Upload Image Data
Reset to demo
Overview
Details
For doctors
After go-live
Compare masks
How to use
Compare aorta segmentations
Built-in synthetic example. No real scans, no API, no sign-in.
Demo only — preset score; no model was run.
Input image
demo-ct.png
Synthetic example
Change
Vendor segmentation
demo-vendor-segmentation.png
Synthetic example
Change
Compare segmentations
Result
High agreement (demo)
Dice 0.940 · 94% agreement
Preset score for the built-in Demo case A pair. This number is looked up, not computed from the images.
Vendor outline
Comparison-model outline
Input image
Synthetic example
Vendor segmentation
Synthetic example
Comparison overlay
Synthetic example
All three panels are synthetic examples. No model was run and no real scan was used. This is an illustrative demo, not a validation result.
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How to use


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Upload Image Data
Reset to demo
Overview
Details
For doctors
After go-live
Compare masks
How to use
How to use Slicepoint
Run the notebook, upload the result, read the answer.
1
What Slicepoint does
Slicepoint checks two things. It checks whether our hospital's scans look like the scans the AI was built and checked on, and it checks how well the AI did on our own scans.
2
What you need
Slicepoint does not read raw scans. It needs two kinds of results, and both are made by our analysis notebooks.
Hospital data
Made from 4 files per hospital:
features.parquet who the patient is and how they were scanned.
labels.parquet the hospital's own aneurysm measurement and label.
predictions.parquet what the AI said.
embeddings.parquet a numeric summary of each image.
All 4 files are linked by case_id.
Image data
Made from the CT images and the aorta outlines. It gives a Dice score: how much our outline and the AI's outline overlap.
3
Step by step
HOSPITAL DATA
STEP 1
The 4 parquet files from your hospital
STEP 2
Run the "Test your hospital" notebook in Colab
STEP 3
Download <hospital>_slicepoint.json, then click "Upload Hospital Data"
IMAGE DATA
STEP 1
The CT images and the aorta outlines
STEP 2
Run the segmentation notebook
STEP 3
Download <hospital>_dice.json, then click "Upload Image Data"
MONTHLY
STEP 1
Each month's files
STEP 2
Run the monthly notebook
STEP 3
Download <hospital>_monthly_summary.json, then upload it on the After go-live tab
4
Why you can't upload the parquet files directly
The checks need Python tools to run, and the scan images are very large. Both jobs run inside the notebook. Only the results come here, as a small JSON file.



5 Keep names matching

Both files for one hospital must use the same hospital name. If the names don't match, Slicepoint shows them as two different hospitals.

6 Privacy

Your files stay in this browser. Nothing is sent anywhere. Scan IDs are shortened. Never upload files that contain patient names.

7 If something goes wrong

What you see	What to do
"This file isn't in the Slicepoint format"	You picked the wrong file. Pick the JSON file the notebook made, not the parquet files.
Two hospitals with the same name in the list	The two files for one hospital used different names. Rename one so both match, then upload it again.
A part is shown as "not loaded"	That dataset has not been uploaded yet. Upload the file for that part.

8 Small glossary

AAA
An abdominal aortic aneurysm. A weak, bulging part of the main artery in the belly.

Contrast
A dye given before the scan. It makes the blood vessels show up brighter.

Slice thickness
How thick each slice of the scan is. Thinner slices show smaller details.

Dice
A score from 0 to 1 for how much two outlines overlap. 1 is a perfect match.

Sensitivity
Of the aneurysms that were really there, how many the AI found.

Specificity
Of the scans that were normal, how many the AI correctly called normal.

Results come from a test sample, not a full trial. This report supports a decision and is not medical advice.

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Data model and API surface

There is no backend data model to replace. The app uses no Base44 entities; all computation happens in Colab notebooks, and the browser only renders JSON the user uploads. Base44 calls observed are limited to analytics, page-view logging and logo hosting.

Pipeline per hospital:

1. **Hospital data.** Four parquet files joined on `case_id`: `features.parquet` (demographics, scanner, contrast, slice thickness), `labels.parquet` (hospital's own diameter and aneurysm label), `predictions.parquet` (model output), `embeddings.parquet` (per-image numeric summary). The "Test your hospital" notebook emits `<hospital>_slicepoint.json`.

2. **Image data.** CT volumes plus reference aorta masks go through the segmentation notebook, which emits `<hospital>_dice.json` (mean Dice; median and share below 0.90 are optional and missing in the demo).
3. **Monthly.** The monthly notebook emits `<hospital>_monthly_summary.json` for the After go-live tab.
4. **Join.** The two hospital files are matched by hospital name string. Mismatched names show up as two hospitals.

What the rebuild needs to define:

- [ ] A versioned JSON schema for each of the three payloads. The POC's schema is implicit in the notebooks, which we have not seen. Ask the team for the notebooks and one sample of each JSON.
- [ ] A stable hospital/site ID instead of name matching.
- [ ] Where the reference (training) distributions come from. In the demo, Hospital A is compared with itself, which is why every check scores 100.
- [ ] Whether `embeddings.parquet` feeds a check. Nothing on screen uses it yet; it reads like a planned covariate-shift test.
- [ ] Persistence. Today nothing is stored; files stay in the browser tab. A FORGE version needs an audit trail for any GO verdict.

Domain logic

The verdict is GO when all five checks meet their targets. That rule is inferred from the copy ("All 5 checks met their targets"); the NO-GO and borderline paths were never shown because the demo data is self-referential.

Check	Score 0 to 100 means	Target	Sub-factors
Different people	Scans per distinct person (leakage guard)	90	Scans, people
Scan settings match	Similarity of acquisition mix to reference; card shows the lowest sub-factor	70	Scanner brand, contrast timing, slice thickness
Patients match	Similarity of patient mix to reference; lowest sub-factor shown	70	Sex, age group, aneurysm size class

Check	Score 0 to 100 means	Target	Sub-factors
Aneurysm labels follow the 30 mm rule	Share of hospital labels consistent with a 30 mm diameter threshold	90	Hospital's inferred cut point
AI accuracy	Composite; card names "outline score" (Dice) as lowest	88	Dice, size within tolerance, aneurysms caught

Fixed constants in the UI:

- Dice markers at 0.81 (basic) and 0.90 (clinical).
- Size tolerance 3 mm by default, user-adjustable on the accuracy tab; Bland-Altman summary uses 2 mm.
- Clinical decision lines at 30 mm and 55 mm; "nearby" means within 5 mm.
- Subgroup bars use a 90% line. Groups below a minimum size print "Too few". The demo suppresses n of 11 or less and shows n of 14 or more, so the cut-off sits between 12 and 14.

Statistics, per the app's own methods statements:

1. Reference standard is the hospital's own labels and measurements, not an independent read.
2. Primary sensitivity and specificity count a missing AI answer as wrong; a secondary "when answered" pair excludes them.
3. 95% Wilson score intervals throughout.
4. Agreement uses AI minus reference; SD divides by n (population SD), not n minus 1.
5. Size class confusion matrix over none / 30 to 54 / 55 or more; balanced accuracy and G-mean reported.
6. Stratified table per group: n scans, n people, bias, MAE, share within tolerance, sensitivity.

`/compare-masks` holds no real logic: it draws synthetic shapes on canvas, overlays them and shows a preset Dice. Rebuild it only if we wire in a real mask comparison.

Open questions for the team: the formula behind each 0 to 100 score (distribution distance used, how sub-factors combine); why the accuracy target is 88 when the Dice clinical line is 0.90; the exact minimum group size.

Design tokens

There is nothing custom to preserve. The stylesheet is the stock shadcn/ui neutral theme: grayscale HSL tokens, Inter for every role, 0.5rem radius, the default five-color chart palette, and a dark mode that is defined but never toggled. Use the AshMatics design system instead.

Token	Light	Dark
background / foreground	`0 0% 100%` / `0 0% 3.9%`	`0 0% 3.9%` / `0 0% 98%`
primary	`0 0% 9%`	`0 0% 98%`
muted-foreground	`0 0% 45.1%`	`0 0% 63.9%`
border	`0 0% 89.8%`	`0 0% 14.9%`
destructive	`0 84.2% 60.2%`	`0 62.8% 30.6%`
chart-1 to chart-5	`12 76% 61%`, `173 58% 39%`, `197 37% 24%`, `43 74% 66%`, `27 87% 67%`	`220 70% 50%`, `160 60% 45%`, `30 80% 55%`, `280 65% 60%`, `340 75% 55%`
radius	`0.5rem`	same
font	Inter, system fallback	same

The parts worth keeping are patterns rather than tokens: the Ours vs reference paired bar, the score bar with a thin target tick, the Dice gauge with two reference markers, and the "What this means" plus "Show the numbers" card footer.

Rework notes

The product idea is sound and the clinical framing is good; the build is a static report viewer with hard-coded thresholds. Keep the question, the five checks and the two-audience layout. Rebuild everything else.

What to keep:

- Verdict-first overview with drill-down, and a separate technical tab that states its methods and limits.

- Missing-counts-as-wrong as the primary metric, decision-line analysis at 30 and 55 mm, and small-group suppression. These are the clinically literate parts.
- Leakage check (scans vs people) as a gating check.

What to change:

- **Compute server-side.** Move notebook logic into a Python service with a versioned schema, so results are reproducible and auditable instead of living in someone's Colab.
- **Make thresholds configuration.** Dice markers, tolerance, decision lines, targets and minimum group size should be per-model, per-site settings with provenance, not constants. The 30 mm rule is AAA-specific; the checks generalize to other imaging AI.
- **Fix the reference.** Comparisons need the vendor's actual training/validation distributions, versioned with the model. Self-comparison makes every check pass.
- **Score transparency.** Each 0 to 100 score needs a published formula and a CI or sample-size warning. Today a score of 100 on 9 aneurysms looks as solid as 100 on 900.
- **Use the embeddings.** An embedding-space shift test would catch covariate shift that categorical metadata misses.
- **Add NO-GO and borderline states** with named failing sub-factors and recommended mitigations.
- **Persist and sign.** Store each assessment, who ran it, model and data versions, and the verdict. This is where it plugs into AI-Watch for the after-go-live monitoring.
- **Plain-language copy is good** but templated phrasing breaks at the edges ("100 of every 100 scans", "reads 0 mm smaller"). Have a clinician edit it.

Next steps:

- [ ] Written MIT grant from the student team
- [ ] Get the three notebooks and a sample of each JSON
- [ ] Decide whether this lands as an AI-Select pre-deployment check, an AI-Watch monitor, or both